

Comprehensive Health & Medical Profile (Master)

Identifying Information

- Name: Kol
 - Age: 26
 - Pronouns: They/She
 - AFAB | Black | Queer | Nonbinary | Polyamorous
 - Location: Kansas City, MO (primary care systems: Truman Medical Centers & Saint Luke's)
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Core Diagnoses (Confirmed)

Connective Tissue & Musculoskeletal

- **Ehlers-Danlos Syndrome – Hypermobile Type (hEDS / Type III)**
- Generalized joint hypermobility
- Recurrent subluxations/dislocations
- Chronic musculoskeletal pain
- Hypotonia
- Poor proprioception
- Early-onset arthritis
- TMJ dysfunction
- Hip dysplasia
- Pes planus (flat feet)
- Poor wound healing, easy bruising, atrophic scarring

- **Seronegative Rheumatoid Arthritis / Inflammatory Arthritis**

Autonomic & Cardiovascular

- **Postural Orthostatic Tachycardia Syndrome (POTS)**
- Orthostatic intolerance
- Tachycardia upon standing
- Syncope / near-syncope
- Heat intolerance
- Blood pooling

Neurological

- Migraines (including visual migraines / aura)
- Brain fog
- Dissociation episodes
- Sleep disturbances

Respiratory

- Asthma

Gastrointestinal

- Chronic GI dysmotility
- GERD
- Nausea
- Constipation (requires daily management)

Psychiatric / Neurodevelopmental

- **Complex PTSD (C-PTSD)** – medical trauma, abandonment trauma
- **AuDHD** (Autism Spectrum + ADHD)
- Depression
- Anxiety
- Emotional dysregulation
- Sensory processing differences
- Rejection Sensitivity Dysphoria (RSD)

Other

- Chronic pain syndrome
- Neurodivergence
- Trauma history (medical + interpersonal)

Functional Impact

- Unable to work consistently or safely
- Requires pacing and frequent rest
- High fall risk during POTS episodes
- Limited standing tolerance
- Cognitive impairment during flares
- Requires assistance with ADLs during bad days

Medication List (Current)

Daily / Scheduled

- Gabapentin – pain
- Propranolol – POTS / tachycardia
- Midodrine – POTS
- Methotrexate – inflammatory arthritis (weekly)
- Folic Acid – arthritis support (not same day as methotrexate)

- Duloxetine – depression / anxiety / pain
- Pantoprazole – GI
- Famotidine – GI
- Vitamin D3
- Symbicort – asthma

As Needed

- Albuterol – asthma
- Ondansetron – nausea
- Cyclobenzaprine – muscle spasms
- Meloxicam – pain/inflammation
- Naltrexone (low dose) – pain modulation
- Topiramate – PTSD / sleep
- MiraLAX – constipation
- Medical cannabis – pain & nausea

Physical Therapy & Mobility

- Daily home PT program
- Balance training
- Core stabilization
- Joint protection focus
- Requires supervision during flares

POTS Episode Identification

Symptoms

- Sudden dizziness
- Tachycardia (+30 bpm on standing)
- Visual changes (static, halos, blind spots)
- Pallor
- Slurred speech
- Weakness / collapse

Immediate Care Protocol

1. Sit or lie down immediately
2. Elevate legs
3. Hydration with electrolytes
4. Cool environment
5. Monitor vitals
6. Gradual re-standing

Emergency Protocol

- Calm tones only
 - No rushing
 - Ask yes/no questions
 - Provide water
 - Quiet environment
 - Call Quincy or Da'Veon
 - If medical emergency: 911 + notify partners
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Care Team

- Primary Care: Amanda Sommerville (Truman)
 - Cardiology: Dr. John Lee (Saint Luke's)
 - Rheumatology: Sarah Ifteqar
 - Pain Management: Fei Cao
 - Gastroenterology: Truman
 - Psychiatry: Dr. Salma Velazquez
 - Therapy: Bridgit Banks
 - Physical Therapy: Kinesphere
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Psychosocial Considerations

- Severe medical gaslighting history
 - Requires trauma-informed care
 - Intersectional discrimination (Black, queer, disabled)
 - High sensitivity to abandonment cues
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Absolute Care Rules

- Believe reported symptoms
 - Never minimize pain
 - Do not attribute symptoms to anxiety
 - Respect autonomy and consent
 - Written instructions preferred
 - Consistency builds safety
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Summary Statement (Use for Systems)

Kol is a multiply disabled, chronically ill, neurodivergent individual with complex multisystem disease requiring comprehensive, trauma-informed, accessible care. Functional limitations are severe, fluctuating, and medically documented. Full disability support is medically necessary for survival and quality of life.